



DISTINCT CHARACTER

Somatic Baseline Practitioner Therapeutic Addendum

Professional implementation, safety, and fidelity guidance

PRACTITIONER-ONLY RESOURCE

Asset ID: DC-P01-SBP-TA01 | Version 1.0 | 2026

RESTRICTED USE

This addendum is available only to approved Distinct Character practitioner-layer users and administrators. It does not grant a professional license, clinical credential, certification, or right to teach, reproduce, adapt, or sublicense the protocol.

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RIGHTS AND SCOPE

Professional Use Notice

This document governs practitioner-supported use of the Somatic Baseline Protocol. It adds implementation boundaries and decision standards; it does not replace the client protocol, professional ethics, supervision, licensure rules, organizational policy, or applicable law.

Purpose

The practitioner layer exists to help an appropriately qualified professional preserve client choice, monitor capacity, recognize limits, and translate protocol observations into proportionate next steps. The central standard is not intensity. It is accurate, ethical, and useful engagement.

Non-treatment boundary

Distinct Character protocols are proprietary educational and behavioral-governance resources. They are not standalone medical or mental-health treatment, diagnostic instruments, crisis services, or substitutes for individualized care. Licensed professionals may integrate selected material only within their existing competence and scope.

Rights boundary

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IMMEDIATE SAFETY

If a client presents with imminent danger, inability to maintain safety, medical emergency, severe disorganization, or a condition requiring mandated action, stop protocol work and follow the emergency, safeguarding, reporting, consultation, and referral procedures required by your role and jurisdiction.

DOCUMENT MAP

Contents

The addendum is organized around the decisions a practitioner must make before, during, and after using Somatic Baseline material with another person.

- Professional scope and qualification boundary
- Readiness, consent, and risk classification
- Practitioner implementation architecture
- Section-by-section clinical adjacency guidance
- Pacing, titration, and adverse-response decisions
- Relational, cultural, and systemic considerations
- Documentation, outcome review, and fidelity
- Printable practitioner tools
- Completion, transition, and references

How to use this addendum

Read the scope, readiness, and adverse-response sections before practitioner-supported use. Use the implementation pages during planning and the printable tools during documentation, supervision, and completion review.

FRAMEWORK LANGUAGE

Terms such as somatic stability, governance, zones, and the SDI belong to the Distinct Character framework. Use them as practical organizing language. Do not present them as established diagnoses, biomarkers, or universally accepted neurophysiological categories.

PART I

Professional Scope and Qualification Boundary

The right use of the material depends on the practitioner's actual role, competence, setting, and legal authority. Practitioner-layer access does not broaden any of those boundaries.

Role	Permitted use	Boundary
Licensed mental health or healthcare professional	May integrate the protocol within the professional's existing scope, assessment process, treatment plan, documentation standards, and applicable law.	The addendum does not expand licensure, replace clinical judgment, or establish a new treatment modality.
Coach, educator, consultant, or facilitator	May support education, reflection, habit formation, environmental review, and non-clinical accountability.	Must not diagnose, treat mental illness, represent the protocol as therapy, manage crises, or advise a client to disregard licensed care.
Distinct Character practitioner-layer user	May use approved portal resources with clients who have appropriate access and within the user's verified professional role.	Portal access is not a license, certification, clinical credential, or authorization to reproduce or teach the intellectual property.

Minimum operating standards

- Use the protocol only with informed, voluntary participation and a clear explanation of your role.
- Remain within competence. Seek supervision or consultation when uncertainty affects safety or quality.
- Maintain confidentiality, secure records, and role-appropriate consent and documentation.
- Use referral pathways that are current, accessible, culturally responsive, and appropriate to the client's location.
- Do not claim guaranteed outcomes, biological correction, trauma resolution, or clinical efficacy from protocol completion.
- Do not use dependency, urgency, fear, or practitioner authority to pressure continuation.

PART II

Readiness, Consent, and Risk Classification

A client may be interested in the work and still not be ready for a specific exercise, intensity, or practitioner relationship. Readiness is assessed repeatedly, not once.

Review area	Proceed when	Pause, adapt, or refer when
Immediate safety	The client can maintain basic safety and participate voluntarily.	There is imminent danger, self-harm risk, violence risk, severe impairment, or inability to maintain safety.
Current stability	The client can orient to the present, communicate choices, and recover from mild activation with support.	The client is acutely intoxicated, manic, psychotic, severely dissociated, medically unstable, or repeatedly unable to reorient.
Environment	The work can occur without increasing material, relational, housing, workplace, or physical danger.	Boundary changes or disclosure could provoke retaliation, coercion, abuse, financial harm, or loss of essential support.
Professional fit	The client's needs fall within the practitioner's competence and role.	The presentation requires trauma treatment, medical assessment, crisis response, substance-use treatment, or another service outside scope.
Consent and choice	The client understands the educational purpose, can decline any exercise, and knows how to stop.	Participation is coerced, confused with treatment, or tied to consequences that remove meaningful choice.

RISK DECISIONS

Green, Amber, and Red Response Levels

Use the simplest classification that supports timely action. These levels organize practitioner response; they do not diagnose a condition.

Level	Observable conditions	Practitioner response
Green - workable engagement	Mild activation; intact orientation; clear speech; client can choose, pause, and recover.	Continue at a measured pace. Track state before and after. End with an ordinary-life next step.
Amber - reduced capacity	Escalating distress, narrowing attention, confusion, marked shutdown, dizziness, panic sensations, or repeated loss of choice.	Stop the exercise. Orient to the room, reduce intensity, return to neutral activity, and decide whether to end. Consult or refer when the pattern repeats.
Red - urgent safety concern	Imminent self-harm or harm risk, medical emergency, severe disorganization, inability to maintain safety, or disclosure requiring mandated action.	Follow emergency, safeguarding, reporting, and organizational procedures for the practitioner's jurisdiction and role. Do not continue protocol work.

Informed participation checklist

- The client understands the educational purpose and the practitioner's role.
- The client knows that any prompt or exercise may be declined or stopped without penalty.
- The client understands confidentiality and its limits in the practitioner's setting.
- The client knows what to do if delayed distress occurs after the session.
- The practitioner has a role-appropriate plan for consultation, referral, safeguarding, and emergencies.
- Remote work includes a reasonable plan for location-specific support when the practitioner's role requires it.

TOUCH AND PHYSICAL DIRECTION

No touch is required to use this protocol. Practitioners must not introduce touch, manual manipulation, restraint, or physical treatment unless it is independently permitted by their profession, clinically appropriate, explicitly consented to, and governed by applicable standards.

PART III

Practitioner Implementation Architecture

The session structure keeps the work narrow enough to monitor and concrete enough to integrate. One useful observation and one governed next step are preferable to an intense but poorly contained experience.

Stage	Practitioner task	Completion signal
1. Frame	Clarify purpose, limits, consent, confidentiality, and the client's right to stop.	The client can explain what the session is and is not.
2. Baseline	Observe current capacity using client language, ordinary behavior, and a brief 0-10 activation rating.	The practitioner has a usable starting point without diagnosing from a proprietary score.
3. Select	Choose one narrow target and the lowest sufficient intensity.	The target is specific, present-focused, and inside scope.
4. Practice	Guide one exercise or reflection while monitoring orientation, breath, speech, movement, choice, and distress.	The client remains able to choose, describe, and pause.
5. Integrate	Translate the observation into one decision, boundary, environmental change, or repeatable practice.	The client leaves with a concrete next action.
6. Close	Reassess state, document response, confirm support, and identify escalation criteria.	The client is at or near baseline and knows what to do if distress rises later.

The observation standard

Document what can be observed or what the client reports: changes in speech, attention, movement, breathing comfort, orientation, choice, distress, recovery time, and daily function. Separate observation from interpretation. A behavior may have multiple causes, including fatigue, pain, medication, disability, culture, environment, neurodivergence, grief, illness, or ordinary preference.

The authority standard

The protocol should increase the client's capacity to notice, choose, and act. When the practitioner becomes the final authority on what the client's body means, the method has moved away from self-mastery. Offer hypotheses lightly, invite correction, and return decisions to the client whenever safety allows.

PART IV

Section-by-Section Clinical Adjacency Guidance

The client protocol uses accessible shorthand to support learning. The practitioner is responsible for preserving uncertainty, avoiding overclaiming, and recognizing when another explanation or service is more appropriate.

Protocol element	Practitioner use	Interpretive limit
Somatic Dysregulation Index (SDI)	Use as a proprietary self-report conversation starter and repeated personal tracking tool.	It is not a validated diagnostic instrument. Scores do not establish a disorder, trauma history, treatment need, or clinical severity.
Zone language	Use Zones 1-3 as accessible descriptions of current capacity and behavioral options.	Do not treat a zone label as a fixed nervous-system state, diagnosis, or objective physiological measurement.
Vagal and polyvagal language	Use as an organizing lens when it helps the client understand regulation, connection, and recovery.	Present polyvagal theory as influential and debated. Avoid claims that a behavior proves a specific vagal pathway or that a brief exercise mechanically resets the vagus nerve.
Environmental Friction Map	Identify conditions that increase load and classify them as remove, restructure, regulate, or refer for specialized support.	Do not individualize structural harm. Personal regulation is not a substitute for safety planning, workplace action, legal support, medical care, or material resources.
Tactical resets	Offer as optional state-management practices. Start with the least activating option and monitor response.	No technique is universally regulating. Stop for pain, dizziness, breathlessness, panic escalation, numbness, or disorientation.
Governance Log	Track triggers, context, choices, recovery time, and functional outcomes to identify patterns.	Avoid compulsive monitoring, moral scoring, or using the log as proof that a client should tolerate an unsafe condition.
Fawn-pattern inquiry	Explore appeasement, over-functioning, conflict avoidance, and self-abandonment as context-dependent adaptive behavior.	Fawn is descriptive framework language, not a DSM diagnosis or a universal female response. Assess power, culture, safety, and consequences before boundary experiments.

EVIDENCE CALIBRATION

What the Practitioner Must Not Overstate

Practical language is useful only when it remains honest about the limits of current evidence and the limits of what can be inferred from an individual response.

Breathing

Slow or exhale-focused breathing may help some people reduce arousal, but responses vary. It should not be described as guaranteed vagal activation, a universal treatment, or proof of nervous-system repair.

Heart-rate variability

HRV is influenced by respiration, posture, fitness, age, medication, illness, measurement conditions, and other factors. Consumer wearables and single readings are not diagnostic.

Polyvagal theory

The framework is influential in clinical practice and continues to be debated in anatomy, measurement, and interpretation. Use it as a model, not settled proof of why a client's behavior occurred.

Interoception

Internal sensations can provide useful information, but they are not automatically accurate explanations of external events. Combine body data with context, evidence, health information, and reflective judgment.

Somatic practices

Evidence differs across practices, populations, and outcomes. A client's immediate felt shift does not establish long-term efficacy, mechanism, or clinical treatment effect.

Identity and fawn patterns

Avoid reducing complex behavior to biology or gender. Analyze history, culture, incentives, coercion, relationships, access, and material consequences.

PART V

Pacing, Titration, and Pendulation

Titration and pendulation are pacing concepts, not proof that activation is therapeutic. The working question is whether the client retains enough orientation, choice, and recovery capacity for the current dose.

Signal	Meaning for pacing	Next move
Client remains oriented and curious	Current dose appears workable.	Continue briefly, then integrate before adding intensity.
Speech speeds up, narrows, or becomes hard to follow	Activation may be outrunning reflective capacity.	Slow the pace; reduce questions; orient to immediate surroundings.
Blankness, collapse, distant gaze, or loss of words	The client may have reduced access to choice or present-moment processing.	Stop content exploration; use neutral orientation; assess safety and whether to end.
Breathing practice causes dizziness or air hunger	The exercise is not currently tolerable or may be medically inappropriate.	Return to normal breathing. Do not coach through symptoms. Refer for medical guidance when indicated.
Neck or body movement causes pain, tingling, weakness, or vertigo	Movement is contraindicated until appropriately assessed.	Stop movement. Use a non-movement option and recommend relevant medical evaluation.
Client repeatedly requests reassurance or permission	The process may be shifting authority away from the client.	Return choice, name options, and ask what information would support an autonomous decision.

Low-capacity adaptation

When capacity is reduced, shorten the session target, use present-focused language, remove performance expectations, and prioritize ordinary functioning. A complete low-capacity session may consist of naming one signal, stopping one source of load, and choosing one support.

DO NOT PUSH THROUGH

Panic, significant breathlessness, dizziness, pain, numbness, escalating unreality, confusion, or loss of choice are reasons to stop and assess. They are not evidence that the exercise is reaching a deeper layer.

ADVERSE RESPONSE

Stop, Orient, Assess, and Escalate

A clear response pathway protects the client and the practitioner from improvising under pressure.

Step	Action
1. Stop	End the exercise or inquiry immediately. Do not frame persistence as courage or progress.
2. Orient	Invite neutral observation of the room, date, location, stable surfaces, and ordinary sensory information. Do not force eye contact or breath control.
3. Assess	Determine whether the client can communicate, choose, maintain safety, and return toward ordinary functioning.
4. Stabilize	Reduce stimulation, offer water or a neutral pause when appropriate, and return to the client's established supports.
5. Escalate	Use emergency, safeguarding, clinical consultation, or referral pathways when symptoms exceed scope or safety cannot be maintained.
6. Document	Record the observable response, action taken, consultation or referral, and follow-up plan without speculative diagnosis.
7. Review	Before future use, reconsider fit, dose, informed consent, practitioner competence, and whether the protocol should be discontinued.

After-session contact

Do not promise availability beyond the boundaries of your service. Provide role-appropriate instructions before the session ends. If delayed symptoms emerge, determine whether the response belongs in routine follow-up, consultation, clinical referral, urgent assessment, or emergency response.

PART VI

Relational, Cultural, and Systemic Considerations

Somatic information occurs inside power, culture, history, disability, economics, and relationships. Ethical use requires more than helping a person tolerate what harms her.

Dimension	Practitioner inquiry	Avoid
Race and culture	How do racialized expectations, surveillance, code-switching, family norms, or community belonging shape the pattern?	Calling protective adaptation pathology or insisting on disclosure that increases risk.
Gender and identity	How do gendered labor, safety calculations, identity threat, and social penalties affect available choices?	Treating women as biologically uniform or presenting appeasement as an innate female trait.
Disability and neurodivergence	Could sensory load, pain, fatigue, communication style, executive function, or access needs explain the response?	Interpreting disability-related needs as resistance, avoidance, or dysregulation.
Economic and workplace power	What material consequences limit the client's ability to remove or confront a stressor?	Prescribing boundaries without considering income, housing, insurance, caregiving, or retaliation.
Faith and community	What beliefs, practices, and communities support regulation, meaning, and belonging?	Replacing a client's worldview with the practitioner's framework language.

Boundary experimentation

Begin with contexts where the client has meaningful choice and manageable consequences. Before encouraging direct confrontation, disclosure, reduced appeasement, or withdrawal of labor, review likely retaliation, dependence, employment, housing, immigration, custody, caregiving, health, and community consequences.

Regulation is not compliance

A calmer presentation is not automatically a better outcome. The client may need anger, grief, distance, protest, legal information, advocacy, rest, medical care, or material change. Regulation should expand response options, not make a person easier for a harmful system to manage.

PART VII

Documentation, Outcome Review, and Fidelity

Documentation should show what occurred, what decision was made, and why the next step was proportionate. It should not convert framework language into unsupported clinical fact.

Outcome domain	Useful evidence	Do not infer
Recovery	Shorter return time after activation; more reliable use of supports.	That the client is cured, trauma-free, or physiologically optimized.
Choice	More ability to pause, decline, request time, or choose a response.	That every difficult emotion should be regulated away.
Function	Improved sleep routine, concentration, communication, follow-through, or daily stability as reported by the client.	Medical improvement without appropriate measurement and clinical evaluation.
Environment	Documented removal, restructuring, or containment of a repeat stressor.	That internal practice can compensate for an unsafe system indefinitely.
Self-trust	The client uses her own observations to make proportionate decisions.	That agreement with the practitioner proves progress.

Documentation rules

- Use the client's own words where meaning is important.
- Record observable signs and reported experience separately from practitioner interpretation.
- Document consent, pauses, declined exercises, adverse responses, consultation, referrals, and safety actions.
- Avoid writing that the SDI diagnosed dysregulation or that a zone label proved a biological state.
- Store records according to professional, contractual, privacy, retention, and breach-response requirements.
- Use only the minimum necessary personal information inside the portal's practitioner note fields.

PRINTABLE TOOL 1

Practitioner Readiness and Scope Review

Complete before beginning practitioner-supported use and revisit whenever client needs, risk, setting, or practitioner role changes.

Practitioner name, role, credential if applicable, and jurisdiction

Client identifier and protocol access confirmed

Purpose of practitioner-supported use

Needs that fall within my competence and role

Known medical, mental-health, accessibility, communication, or environmental considerations relevant to pacing

Current referral, consultation, safeguarding, and emergency pathways

Consent and confidentiality discussion completed on

PRINTABLE TOOL 2

Session Preparation Record

Use this page to define the smallest useful scope before the session begins.

Session date, location or delivery format, and planned duration

Client-defined objective for this session

Baseline activation or capacity rating and client language

Protocol section or tool selected

Why this selection is proportionate today

Stop signals and adaptations agreed in advance

Ordinary-life integration target

PRINTABLE TOOL 3

Practitioner Observation Record

Record what was observed and reported without converting the observation into a diagnosis.

Observation point	Before	During	After
Orientation and attention			
Speech and communication			
Movement and posture			
Breathing comfort			
Choice and consent			
Reported distress or comfort			
Recovery and daily function			

Client's own words about the experience

Practitioner interpretation, clearly labeled as a working hypothesis

Action, adaptation, consultation, or referral

PRINTABLE TOOL 4

Adverse Response and Referral Record

Complete when an exercise is stopped for distress, loss of orientation, physical symptoms, risk, or a scope concern.

Date, time, setting, and activity underway

Observable response and client's report

Exercise stopped at

Orientation or stabilization actions used

Safety assessment and outcome

Consultation, referral, safeguarding, or emergency action

Follow-up responsibility, timing, and communication

Changes required before any future protocol use

PRINTABLE TOOL 5

Supervision and Consultation Record

Use consultation to improve decisions, not merely to obtain reassurance after the fact.

Question requiring consultation

Relevant facts, client preferences, risk, and context

What is inside and outside my role

Consultant or supervisor and date

Options considered

Decision and rationale

Client communication and follow-up

PRINTABLE TOOL 6

Fidelity Review

Use at midpoint, completion, and whenever the process feels stuck or practitioner-led rather than client-governed.

Review: The client retains meaningful choice and can decline or stop without penalty.

Status: Clear / Needs attention Evidence or correction: _____

Review: Protocol language is presented as framework language rather than diagnosis or settled mechanism.

Status: Clear / Needs attention Evidence or correction: _____

Review: The selected exercise is inside my competence and proportionate to current capacity.

Status: Clear / Needs attention Evidence or correction: _____

Review: I am not using regulation to encourage tolerance of an unsafe or exploitative condition.

Status: Clear / Needs attention Evidence or correction: _____

Review: Client observations and practitioner interpretations remain distinct in documentation.

Status: Clear / Needs attention Evidence or correction: _____

Review: Progress is evaluated through function, choice, recovery, and structural change rather than compliance with me.

Status: Clear / Needs attention Evidence or correction: _____

Review: Referral, consultation, and escalation have occurred when needs exceeded scope.

Status: Clear / Needs attention Evidence or correction: _____

Review: The client can describe how she will continue without dependence on the practitioner.

Status: Clear / Needs attention Evidence or correction: _____

PART VIII

Completion and Transition

Practitioner-supported use is complete when the client can recognize relevant signals, choose proportionate actions, and identify when another kind of support is needed.

Completion review

- The client can distinguish observation from interpretation.
- The client can identify at least two usable supports and the conditions under which each should be used.
- The client can name stop signals and escalation criteria.
- At least one source of environmental friction has been removed, restructured, or addressed through an appropriate external pathway.
- The client has a realistic continuation plan that does not require constant practitioner access.
- Outstanding clinical, medical, legal, safety, or material needs have been referred or clearly documented.

Transition note

Completion of Somatic Baseline does not require the absence of stress or difficult emotion. It requires a more accurate relationship to capacity, a safer method of responding to state changes, and clearer decisions about which problems need internal practice and which require structural or professional intervention.

FINAL FIDELITY STANDARD

The practitioner succeeds when the client leaves with more authority, more accurate choices, and less dependence on the practitioner's interpretation.

EVIDENCE AND CONTEXT

Selected References

These sources support trauma-informed principles, evidence calibration, and professional caution. They do not validate the Distinct Character framework or the SDI as clinical instruments.

1. Substance Abuse and Mental Health Services Administration. Trauma-Informed Care in Behavioral Health Services (TIP 57). Updated 2025. <https://www.samhsa.gov/resource/dbhis/tip-57-trauma-informed-care-behavioral-health-services>
2. Substance Abuse and Mental Health Services Administration. Trauma-Informed Approaches and Programs. Updated 2026. <https://www.samhsa.gov/mental-health/trauma-violence/trauma-informed-approaches-programs>
3. National Institute for Health and Care Excellence. Post-traumatic stress disorder: NG116. Published 2018; reviewed 2025. <https://www.nice.org.uk/guidance/ng116>
4. Balban, M. Y., et al. (2023). Brief structured respiration practices enhance mood and reduce physiological arousal. *Cell Reports Medicine*, 4(1), 100895. <https://pubmed.ncbi.nlm.nih.gov/36630953/>
5. Laborde, S., Mosley, E., & Thayer, J. F. (2017). Heart Rate Variability and Cardiac Vagal Tone in Psychophysiological Research. *Frontiers in Psychology*, 8, 213. <https://pubmed.ncbi.nlm.nih.gov/28265249/>
6. Quigley, K. S., et al. (2021). Functions of Interoception: From Energy Regulation to Experience of the Self. *Trends in Neurosciences*, 44(1), 29-38. <https://pubmed.ncbi.nlm.nih.gov/33378654/>
7. Kuhfuss, M., et al. (2021). Somatic experiencing: effectiveness and key factors of a body-oriented trauma therapy. *European Journal of Psychotraumatology*, 12(1), 1929023. <https://pubmed.ncbi.nlm.nih.gov/34290845/>
8. Manzotti, A., et al. (2024). An in-depth analysis of the polyvagal theory in light of current findings in neuroscience and clinical research. *Developmental Psychobiology*, 66(2), e22450. <https://pubmed.ncbi.nlm.nih.gov/38388187/>

Version history

Version	Date	Change
1.0	August 2026	Initial practitioner-only therapeutic addendum created and activated for the Distinct Character portal.

Somatic Baseline Practitioner Therapeutic Addendum | DC-P01-SBP-TA01